

ELITE PHYSICIAN WEALTH PLANNING™

Powered by M A Epps & Associates

Advanced Wealth, Tax & Retirement Strategies for Physicians



2026 CURRENT-LAW GUIDE

The Physician Tax & Retirement Planning Guide

A practical planning framework for high-income physicians, specialists, and practice owners who want to reduce unnecessary taxes, increase retirement readiness, and coordinate their financial decisions.

Current-Law Standard

This guide is based on federal tax, retirement, Medicare, payroll-tax, HSA, charitable, student-loan, and estate-planning guidance available as of July 7, 2026. It intentionally focuses on rules that are most relevant to physicians and medical professionals. State-specific rules, practice-specific entity issues, and individual tax return facts require separate review.

Prepared for	Elite Physician Wealth Planning™ lead magnet and client education campaign
Prepared by	M A Epps & Associates
Founder	Michael A. Epps, ChFC®, RICP® - Founder & Chief Wealth Strategist
Version	EPWP v3.3
Status	Draft for compliance and professional review
Date	2026-07-07

Helping Physicians Keep More. Grow More. Retire Better.

Important Notice

Educational Material - Not Individual Tax, Legal, or Investment Advice

This guide is designed for education and planning discussion only. Tax laws, retirement rules, plan documents, state rules, employer benefits, entity structures, and client facts can materially change the analysis. No reader should implement a tax, retirement, estate, investment, business, student-loan, or insurance strategy solely from this guide. Coordinate with a CPA, attorney, plan administrator, and qualified financial professional before acting.

Use this guide to identify planning questions and opportunities. Use professional review to determine what applies to a specific physician, household, medical practice, or retirement plan.

Contents

1. Executive Summary for Physicians
2. 2026 Current-Law Planning Dashboard
3. The Elite Physician Tax Strategy Map
4. Federal Income Tax Planning for High-Income Physicians
5. W-2 Physicians: Planning Levers That Often Get Missed
6. Practice Owners: Entity, Payroll, QBI/SSTB, and Deduction Strategy
7. Retirement Plan Deep Dive: 401(k), 403(b), 457(b), SEP, Profit Sharing, Cash Balance, and Defined Benefit Plans
8. Roth Planning, Backdoor Roth, After-Tax Contributions, and Asset Location
9. HSA, Medical Benefits, and Executive Benefit Planning
10. Charitable Giving, SALT, AMT, and Itemized Deduction Strategy
11. Student Loan and PSLF Planning for Physicians
12. Retirement Income, RMDs, Social Security, Medicare, and IRMAA
13. Estate, Gift, Asset Protection, and Legacy Planning
14. The 90-Day Physician Planning Sprint
15. Checklists, Data Request, and Source Notes

How to use this guide

- Read the current-law dashboard first to understand the major thresholds.
- Use the segment-specific sections for W-2 physicians, practice owners, and physicians nearing retirement.
- Use the 90-day sprint and data request checklist to turn education into a coordinated planning engagement.

1. Executive Summary for Physicians

Physicians often earn high income, but high income alone does not create financial independence. The difference comes from coordinated decisions: how income is taxed, how retirement plans are designed, how cash flow is allocated, how student loans are handled, how risk is protected, and how wealth is eventually distributed.

The central planning challenge is not simply finding deductions. It is building a repeatable annual process that connects tax planning, retirement planning, investment strategy, debt management, practice planning, and legacy planning.

The physician planning gap

- Tax planning is often reactive and happens after the year is over.
- Retirement contributions are sometimes maximized without evaluating tax brackets, Roth opportunities, after-tax contributions, or practice-owner plan design.
- High-income W-2 physicians may miss benefit, withholding, 457(b), HSA, and backdoor Roth coordination opportunities.
- Practice owners may miss larger opportunities in cash balance plans, defined benefit plans, profit sharing, accountable plans, entity structure, and succession planning.
- Retirement-income planning often starts too late, creating avoidable RMD, Medicare IRMAA, and legacy-tax issues.

The Elite Physician Planning Objective

Planning Objective

Help physicians keep more of what they earn, build more for the future, and retire with greater confidence through a coordinated annual process that converts current law into actionable planning decisions.

The five areas that drive most physician tax and retirement outcomes

Planning Area	Why It Matters
1. Income character	Wages, K-1s, S-corp distributions, 1099 income, rental income, capital gains, and retirement distributions are taxed differently.
2. Timing	Income timing, deduction timing, Roth conversions, charitable giving, and retirement distributions can materially affect lifetime tax cost.
3. Retirement plan design	A basic 401(k) may be insufficient for a high-income practice owner; cash balance or defined benefit design may create larger deductions.
4. Tax-surface coordination	Federal brackets, payroll taxes, SALT, NIIT, AMT, QBI, IRMAA, and state taxes interact.
5. Implementation discipline	Tax opportunities often require documents, elections, plan amendments, payroll changes, CPA coordination, or year-end action.

2. 2026 Current-Law Planning Dashboard

This section summarizes federal law items most likely to affect high-income physicians, specialists, and practice owners. The amounts below should be confirmed before implementation because IRS guidance, forms, and state law may change.

2026 tax and retirement thresholds

Current-Law Item	2026 Amount / Rule	Planning Use
Standard deduction	Single/MFS: \$16,100; MFJ: \$32,200; Head of household: \$24,150	Affects itemizing, charitable strategy, SALT value, and taxable-income planning. [S1]
Federal brackets	Top rate remains 37%; top bracket begins above \$640,600 single and \$768,700 MFJ	High-income physicians may benefit from bracket management and timing strategies. [S1]
SALT deduction cap	\$40,400 for 2026 (\$20,200 MFS), phased down above MAGI \$505,000 (\$252,500 MFS), but not below \$10,000 (\$5,000 MFS)	Important for physicians in higher-tax states and for itemizing analysis. [S4]
Estate basic exclusion	\$15,000,000 for estates of decedents dying in 2026	Creates a planning window for high-net-worth physicians and practice owners. [S1]
401(k)/403(b)/457(b) deferral	\$24,500	Baseline salary-deferral planning. [S2]
Age 50+ catch-up	\$8,000 for most 401(k)/403(b)/governmental 457 plans	For physicians age 50+. [S2]
Age 60-63 catch-up	\$11,250 for most 401(k)/403(b)/governmental 457 plans	SECURE 2.0 enhanced catch-up window. [S2]
Defined contribution plan limit	\$72,000	Applies to total annual additions before catch-up contributions. [S3]
Defined benefit plan limit	\$290,000 annual benefit limit	Relevant to cash balance and defined benefit design. [S3]
IRA limit	\$7,500 plus \$1,100 catch-up at age 50+	Backdoor Roth and basis review. [S2]
HSA contribution limit	Self-only: \$4,400; Family: \$8,750	Tax-deductible contribution, tax-deferred growth, tax-free qualified medical distributions. [S5]
Social Security wage base	\$184,500	Payroll tax budget, S-corp compensation, and owner cash flow planning. [S13]
Additional Medicare Tax	0.9% over \$200,000 single/HOH, \$250,000 MFJ, \$125,000 MFS	High-income W-2 and self-employed physicians often trigger this tax. [S9]
Net Investment Income Tax	3.8% on lesser of net investment income or MAGI excess over \$200,000 single/HOH, \$250,000 MFJ, \$125,000 MFS	Relevant to portfolio, rental, passive income, and sale planning. [S10]
Medicare Part B premium	2026 standard monthly premium: \$202.90; high-income IRMAA tiers begin above MAGI \$109,000 single / \$218,000 MFJ	Retirement-income planning should account for two-year lookback income. [S12]
Employer educational assistance	Up to \$5,250 excluded from income in 2025 and 2026 for qualified educational assistance, including qualified student loan payments	May help employed physicians and practice owners design benefits. [S16]

Current-law interpretation rule

- Use these thresholds as planning inputs, not as standalone recommendations.
- Verify state tax rules, plan documents, employer plan features, and client-specific facts before implementation.
- When a law does not apply to a physician segment, omit it from the client-facing recommendation.

3. The Elite Physician Tax Strategy Map

The strongest physician planning engagements start by mapping the tax surface before recommending products or tactics. A doctor with \$600,000 of W-2 income needs a different planning sequence than a surgeon with K-1 practice income, a dentist with an S corporation, or a physician executive approaching Medicare age.

Elite Physician Tax Strategy Map

1. Income Type

W-2 salary, K-1 income, 1099, S-corp wages, rental income, portfolio income

2. Tax Surface

Federal brackets, SALT, payroll tax, NIIT, AMT, QBI/SSTB, state tax

3. Planning Lever

Retirement plans, HSA, charitable giving, Roth, entity design, deductions

4. Implementation

Coordinate CPA, TPA, attorney, payroll, custodian, and plan documents

5. Annual Review

Update before year-end based on income, tax law, practice cash flow, and goals

Planning principle:

Physicians rarely need one isolated tactic. They need a coordinated sequence that manages tax cost, cash flow, retirement savings, and implementation risk in the same planning cycle.

Core diagnostic questions

1. What type of income is being earned, and who controls its timing?
2. Which tax surfaces are triggered: ordinary income, payroll tax, NIIT, SALT, AMT, QBI/SSTB, capital gains, Medicare IRMAA, or estate tax?
3. What planning levers exist before year-end?
4. Which strategies require a CPA, TPA, attorney, payroll provider, custodian, or employer approval?
5. What should be reviewed annually to prevent tax drift?

Advisor Positioning

The planning process should lead with diagnosis. The client should see the relationship among taxes, retirement savings, benefits, and implementation before any investment or insurance recommendation is discussed.

4. Federal Income Tax Planning for High-Income Physicians

Most physicians are affected by a compressed set of planning variables: high ordinary income, limited time, concentrated career risk, substantial payroll taxes, and delayed retirement planning. Federal tax planning should focus on what can be controlled before December 31 and what can be positioned for future years.

Deep dive: bracket management

The 2026 federal rate schedule keeps seven individual income tax brackets, with the highest marginal rate of 37% beginning at income above \$640,600 for single filers and \$768,700 for married couples filing jointly. A high-income physician often cannot avoid the top bracket in a high-income year, but may still manage taxable income through retirement contributions, business deductions, charitable giving, loss harvesting, and timing of bonuses, distributions, and Roth conversions. [S1]

Planning moves to consider

- Compare current-year marginal rate with projected retirement marginal rate before choosing traditional vs Roth contributions.
- Model bunching of charitable deductions when the standard deduction would otherwise reduce the value of annual giving.
- Use multi-year capital gain and tax-loss harvesting rather than waiting until December.
- For practice owners, coordinate major equipment purchases, retirement plan contributions, entity distributions, and owner compensation before year-end.
- For physicians near retirement, model Roth conversions before RMDs, Social Security, and Medicare IRMAA exposure increase.

Deep dive: SALT and itemized deductions

For 2026, the overall deduction limit for state and local income, sales, and property taxes is \$40,400, reduced when modified AGI exceeds \$505,000. The deduction cannot be reduced below \$10,000. Married filing separately amounts are lower. This is especially relevant for high-income physicians in states with higher income and property taxes. [S4]

SALT planning questions

- Will the physician itemize after accounting for SALT, mortgage interest, and charitable giving?
- Will income push the taxpayer into the SALT phase-down range?
- Should charitable gifts be bunched into a donor-advised fund year?
- Does the physician own a pass-through business that may have an entity-level state tax election? State-specific review is required.

Deep dive: AMT awareness

For 2026, the AMT exemption amount is \$90,100 for unmarried individuals and \$140,200 for married couples filing jointly, with phaseout beginning at \$500,000 and \$1,000,000 respectively. AMT review is especially relevant when a physician has incentive stock options, large state tax deductions, unusual itemized deductions, or complex K-1 income. [S1]

Deep dive: payroll tax and investment surtax

Tax Surface	Rule	Planning Use
OASDI Social Security	6.2% employee and employer on wages up to \$184,500; self-employed taxpayers generally pay both halves through SE tax subject to rules.	Budget owner payroll and compare reasonable compensation in S-corp planning. [S13]
Medicare tax	1.45% employee and employer with no wage base cap.	All covered wages are subject to Medicare tax. [S13]
Additional Medicare Tax	0.9% on wages/self-employment income above thresholds.	Often triggered by attending physicians, specialists, and dual-income households. [S9]
NIIT	3.8% on net investment income or MAGI excess over thresholds.	Coordinate investment income, passive rentals, capital gains, and business sale planning. [S10]

5. W-2 Physicians: Planning Levers That Often Get Missed

Many employed physicians assume they have limited planning opportunities because they receive a W-2. The planning tools are different from practice-owner strategies, but they are still significant. The key is to optimize employer benefits, payroll elections, student-loan strategy, retirement savings, and tax-efficient investing.

W-2 physician opportunity checklist

Area	What to Review	When
403(b), 401(k), or TSP	Confirm the plan type, pre-tax/Roth options, match, vesting, and investment lineup.	Annual open enrollment and after contract changes
457(b)	Determine governmental vs non-governmental status, distribution rules, creditor risk, and whether the plan stacks with 403(b)/401(k).	Before maximizing contributions
HSA	Confirm HDHP eligibility, employer contributions, family coverage, and investment options.	Open enrollment and family changes
Backdoor Roth IRA	Evaluate IRA balances, pro-rata rule, spouse IRAs, and Form 8606 tracking.	Before contribution/conversion
Disability insurance	Review own-occupation coverage, group offsets, taxation of benefits, and specialty definitions.	After training, job changes, and income growth
Student loan strategy	Coordinate PSLF/IDR, refinancing, filing status, income certification, and employer assistance.	Residency, fellowship, first attending job, and nonprofit employment
Withholding	Review W-4 for dual-income households, bonuses, K-1s, and investment income.	Midyear and year-end

Deep dive: governmental vs non-governmental 457(b)

A governmental 457(b) can often be rolled to an IRA or another eligible plan after separation, while a non-governmental 457(b) may remain subject to the employer's creditors and have more restrictive distribution elections. The financial value of a 457(b) contribution depends on tax deferral, investment options, distribution risk, and the physician's future tax rate.

Deep dive: employer student-loan assistance

Under current IRS guidance, employer-provided educational assistance can include payments of principal or interest on a qualified education loan for the employee's own education. For 2025 and 2026, the first \$5,250 of qualifying educational assistance benefits can be excluded from income if provided under a qualifying plan. [S16]

W-2 physician action items

- Request a benefits summary and plan documents, not just payroll screenshots.
- Ask whether the employer offers 457(b), HSA, supplemental disability, legal plans, or student-loan assistance.
- Review whether Roth, pre-tax, or mixed salary deferrals fit the physician's career stage and projected retirement tax bracket.
- Track basis and conversions carefully if using backdoor Roth strategies.

6. Practice Owners: Entity, Payroll, QBI/SSTB, and Deduction Strategy

Practice owners have additional planning levers because they may influence entity structure, compensation, retirement plan design, employee benefits, depreciation, and succession planning. The opportunity is larger, but so is the implementation risk.

Entity structure is not tax planning by itself

A medical practice may operate as an LLC, partnership, professional corporation, S corporation, C corporation, or another professional entity depending on state law, ownership, licensing, and legal requirements. The entity choice affects payroll tax, qualified plan design, fringe benefits, basis, distributions, buy-sell agreements, and eventual sale or succession planning.

Deep dive: S corporation planning

S corporation planning should never be reduced to simply “take less salary.” A shareholder-employee must receive reasonable compensation for services. The right compensation level depends on specialty, clinical hours, administrative duties, practice profits, market compensation, retirement plan design, and payroll-tax exposure.

Deep dive: QBI and medical practices

The qualified business income deduction may allow eligible taxpayers to deduct up to 20% of QBI from certain domestic pass-through businesses. However, physicians are typically in health-related specified service trade or business territory, and income limits, wage/property limitations, and form-specific rules can materially reduce or eliminate the deduction for high-income owners. [S7]

For 2026 planning, use current IRS forms and instructions when available. Draft 2026 QBI forms indicate that taxpayers above income thresholds may need Form 8995-A rather than simplified Form 8995, with threshold figures around \$201,750 single and \$403,500 married filing jointly in draft materials. Confirm final forms before relying on exact thresholds. [S8]

Deep dive: equipment, depreciation, and Section 179

Practice owners purchasing equipment should coordinate business need, financing, depreciation, state conformity, and income timing. The IRS has issued guidance under the Working Families Tax Cuts for permanent 100% additional first-year depreciation for eligible depreciable property acquired after January 19, 2025. [S18]

IRS Publication 946 states that for tax years beginning in 2026, the maximum Section 179 expense deduction is \$2,560,000 and begins to phase out when Section 179 property placed in service exceeds \$4,090,000. Certain SUVs have a separate \$32,000 limit. [S19]

Practice owner planning checklist

- Review entity structure annually with CPA and legal counsel.
- Confirm reasonable compensation methodology for S-corp owners.
- Evaluate whether retirement plan design should move beyond basic deferrals into profit sharing, new comparability, cash balance, or defined benefit design.
- Coordinate major equipment purchases with actual clinical need and tax benefit timing.
- Review accountable plan reimbursements, business insurance, lease arrangements, buy-sell agreements, and succession planning.
- Do not implement deductions without documentation, business purpose, and CPA review.

7. Retirement Plan Deep Dive

Retirement planning for physicians should not begin with investment selection. It should begin with contribution capacity, employer plan options, tax rate analysis, plan design, and future retirement-income needs.

Retirement Savings Stack for Physicians

Base: Emergency reserves and disability coverage

Before aggressive savings, protect income and liquidity.

Tier 1: Workplace plan deferrals

Maximize 401(k), 403(b), governmental 457(b), or TSP where appropriate.

Tier 2: Employer match and profit sharing

Capture matching contributions and evaluate employer contributions.

Tier 3: Backdoor Roth / after-tax strategy

Evaluate IRA basis, pro-rata rules, and plan features before use.

Tier 4: HSA

Use as a medical reserve or long-term tax-preferred account when eligible.

Tier 5: Cash balance / defined benefit plan

For practice owners with high income and stable cash flow.

Tier 6: Tax-efficient brokerage and legacy assets

Coordinate capital gains, charitable giving, estate planning, and retirement income.

Decision rule: prioritize tax-adjusted value, liquidity, eligibility, and long-term retirement income - not merely maximum deduction

2026 retirement plan limits most relevant to physicians

Plan Limit	2026 Amount
401(k), 403(b), governmental 457(b), TSP elective deferral	\$24,500
Catch-up age 50+	\$8,000 for most plans
Enhanced catch-up age 60-63	\$11,250 for most 401(k), 403(b), governmental 457(b), and TSP plans
Defined contribution annual additions limit	\$72,000
Annual compensation limit for qualified plans	\$360,000
SEP maximum contribution	\$72,000
SIMPLE IRA deferral	\$17,000
SIMPLE IRA catch-up age 50+	\$4,000; enhanced age 60-63 catch-up generally \$5,250
IRA contribution limit	\$7,500
IRA catch-up age 50+	\$1,100
Defined benefit annual benefit limit	\$290,000
Taxable wage base used in retirement-plan limit tables	\$184,500

Deep dive: cash balance and defined benefit plans

Cash balance and defined benefit plans can be valuable for physicians and dental specialists with high income, stable practice cash flow, and a desire for larger tax-deductible retirement contributions. These plans require actuarial design, annual funding discipline, employee demographics analysis, plan documents, and ongoing administration.

Good fit characteristics

- Owner or partner has consistently high income and wants to accelerate retirement savings.

- Practice cash flow can support required contributions.
- Employee demographics make the plan design cost-effective.
- The owner can commit to multi-year funding and administrative requirements.
- The CPA, TPA, investment advisor, and payroll provider can coordinate execution.

Red flags

- Highly volatile income or uncertain practice profitability.
- No willingness to fund required contributions.
- Employee demographics make the plan disproportionately expensive.
- Owner wants maximum deduction without understanding funding obligations.
- Late-year setup without enough time for plan design and documentation.

Planning Sequence

For practice owners, retirement-plan design should be modeled before year-end, not after tax preparation begins. A cash balance plan may be a tax strategy, a retirement strategy, and an employee-benefits strategy at the same time.

8. Roth Planning, Backdoor Roth, After-Tax Contributions, and Asset Location

Roth planning is less about one contribution and more about managing tax flexibility. A physician may benefit from having assets in three tax buckets: taxable accounts, tax-deferred accounts, and tax-free Roth accounts. This flexibility becomes valuable in retirement when managing RMDs, capital gains, Medicare IRMAA, and survivor taxes.

Backdoor Roth IRA planning

Many high-income physicians earn too much for direct Roth IRA contributions. A backdoor Roth strategy may still be possible, but it requires careful review of pre-tax IRA balances and the pro-rata rule. The IRA contribution limit for 2026 is \$7,500, with a \$1,100 catch-up contribution for individuals age 50 or older. [S2]

Backdoor Roth checklist

- Check all traditional, SEP, and SIMPLE IRA balances before conversion.
- Confirm whether rollover to an employer plan can isolate basis and reduce pro-rata complications.
- Track nondeductible IRA basis on Form 8606.
- Coordinate both spouses separately.
- Avoid assuming the conversion is tax-free without reviewing prior IRA balances.

Roth vs traditional decision framework

Situation	Likely Planning Direction
Current tax rate is unusually high	Traditional may be more attractive if future rates are lower.
Current tax rate is temporarily low	Roth contribution or conversion may be attractive.
Early-career physician with rising income	Roth strategies may be more compelling before peak earning years.
Practice owner seeking cash-flow deduction	Traditional/cash balance contributions may create immediate tax leverage.
Near-retirement physician before RMDs	Partial Roth conversions may reduce future RMD/IRMAA pressure.
Legacy-focused physician	Roth assets may provide estate and beneficiary planning flexibility.

Asset location

Asset location means coordinating which assets are held in taxable, tax-deferred, and Roth accounts. For example, tax-inefficient assets may be more appropriate in tax-deferred accounts, while broad-market equity strategies with favorable capital-gain treatment may be efficient in taxable accounts. The exact answer depends on the portfolio, tax bracket, withdrawal plan, and estate objective.

9. HSA, Medical Benefits, and Executive Benefit Planning

Health Savings Accounts can be particularly useful for high-income physicians because eligible contributions may be deductible, growth can be tax-deferred, and qualified medical distributions can be tax-free. This combination is rare in the tax code.

2026 HSA current-law limits

HSA / HDHP Item	2026 Amount
HSA contribution limit - self-only	\$4,400
HSA contribution limit - family	\$8,750
HDHP minimum deductible - self-only	\$1,700
HDHP minimum deductible - family	\$3,400
HDHP out-of-pocket maximum - self-only	\$8,500
HDHP out-of-pocket maximum - family	\$17,000
Excepted-benefit HRA amount	\$2,200 for plan years beginning in 2026

The IRS also notes that telehealth and other remote-care services can now be received before meeting a high-deductible health plan deductible without disqualifying HSA eligibility, and certain bronze/catastrophic plans and direct primary care arrangements are addressed under updated Working Families Tax Cuts guidance. Confirm plan-specific eligibility before contributing. [S20]

HSA planning paths

- Cash-flow use: contribute and reimburse current qualified medical expenses.
- Long-term accumulation use: contribute, invest if appropriate, and save receipts for future reimbursement.
- Pre-retirement use: build a reserve for Medicare premiums and retirement healthcare costs where eligible.
- Family coordination: review spouse coverage, dependent eligibility, and enrollment timing.

Practice owner benefits review

Practice owners should review group health coverage, disability benefits, life insurance, long-term care planning, employee benefits, educational assistance programs, and retirement plan benefits together. The objective is not merely to add benefits, but to create a compensation structure that supports recruitment, retention, owner planning, and tax efficiency.

10. Charitable Giving, SALT, AMT, and Itemized Deduction Strategy

Many physicians are charitably inclined, but tax-efficient giving requires coordination with the standard deduction, itemized deductions, appreciated assets, donor-advised funds, QCDs, capital gains, and estate goals.

Current law charitable giving rule for non-itemizers

Beginning with tax year 2026, taxpayers who do not itemize may deduct up to \$1,000 (\$2,000 if filing jointly) of cash contributions to certain qualified organizations. Taxpayers who itemize generally use Schedule A subject to applicable limits and substantiation rules. [S6]

Bunching strategy

When annual charitable gifts do not generate an itemized deduction because the standard deduction is higher, physicians may consider bunching multiple years of giving into one year. A donor-advised fund may allow a larger deduction in the contribution year while grants to charities can occur over time. This strategy requires cash-flow review, charitable intent, and documentation.

Appreciated securities

A physician with taxable investments may consider donating appreciated securities held long-term instead of cash. The potential benefit is avoiding capital gain on the donated asset while receiving a charitable deduction if itemizing and otherwise eligible. This is especially relevant after a bull market, concentrated stock position, or practice sale liquidity event.

Qualified charitable distributions

For physicians age 70 1/2 or older, QCDs can be used to send IRA assets directly to eligible charities. QCDs may count toward RMDs and can reduce adjusted gross income when properly executed. Current IRS retirement guidance reflects a 2026 aggregate QCD exclusion limit of \$111,000. [S15]

Charitable planning questions

- Will the client itemize in the current year?
- Are gifts being made in cash, appreciated securities, or retirement assets?
- Will a QCD reduce AGI, RMD exposure, or IRMAA pressure?
- Are substantiation requirements satisfied before the return is filed?
- Does the giving strategy align with estate and family objectives?

11. Student Loan and PSLF Planning for Physicians

Student-loan strategy can materially affect physician cash flow, career decisions, filing status, and retirement savings capacity. The right approach depends on loan type, employer type, income trajectory, specialty, family status, and whether the physician is pursuing forgiveness.

PSLF current framework

Federal Student Aid describes Public Service Loan Forgiveness as forgiving the remaining balance on eligible Direct Loans after 120 qualifying monthly payments under a qualifying repayment plan while working full-time for a qualifying employer. Many physicians employed by government hospitals, academic medical centers, and certain nonprofit hospitals may need to evaluate PSLF eligibility. [S14]

IDR and current court-action awareness

Federal Student Aid reports that a March 10, 2026 court order prevents the Department of Education from implementing the SAVE Plan and parts of other IDR plans, while eligible borrowers can apply for or recertify under IBR, ICR, and PAYE as described by StudentAid.gov. This area is especially fluid and should be checked before making repayment decisions. [S17]

Physician student-loan decision tree

Situation	Planning Focus
Works for nonprofit/government employer	Evaluate PSLF, employer eligibility, Direct Loan status, repayment plan, and annual certification.
Private practice or for-profit employer	Compare refinancing, standard repayment, aggressive payoff, and investing tradeoffs.
Resident/fellow	Coordinate income certification, PSLF count, interest, disability coverage, and early retirement savings.
High-income attending	Model after-tax cash flow before refinancing or accelerating payoff.
Dual-income household	Review filing status, IDR payment effect, spouse loans, and tax cost.
Practice owner	Consider employer educational assistance plan design when recruiting or retaining physicians.

Student-loan action items

- Download loan details from StudentAid.gov before modeling.
- Confirm employer eligibility using the PSLF Help Tool or official employer search.
- Do not refinance federal loans privately until forgiveness, IDR, deferment, and protection tradeoffs are reviewed.
- Coordinate student-loan payments with retirement plan contributions and emergency reserves.

12. Retirement Income, RMDs, Social Security, Medicare, and IRMAA

Retirement-income planning for physicians is tax planning. Decisions around Roth conversions, taxable-account withdrawals, pension elections, Social Security, RMDs, charitable giving, and Medicare premiums can interact over decades.

RMD rules

IRS retirement guidance states that account owners generally must begin taking required minimum distributions from traditional IRAs, SEP IRAs, SIMPLE IRAs, and retirement plan accounts when they reach age 73. Roth IRAs and designated Roth accounts in 401(k) or 403(b) plans generally do not require lifetime RMDs for the owner, though beneficiary rules apply. [S11]

Medicare and IRMAA

For 2026, CMS reports the standard Medicare Part B monthly premium is \$202.90 and the annual Part B deductible is \$283. High-income beneficiaries pay income-related monthly adjustment amounts. In 2026, the first Part B IRMAA tier begins above MAGI of \$109,000 for individual tax returns and \$218,000 for joint returns. [S12]

2026 Part B Full-Coverage MAGI Tier	Total Monthly Part B Premium
Individual MAGI <= \$109,000 / Joint <= \$218,000	\$202.90
Individual > \$109,000 to \$137,000 / Joint > \$218,000 to \$274,000	\$284.10
Individual > \$137,000 to \$171,000 / Joint > \$274,000 to \$342,000	\$405.80
Individual > \$171,000 to \$205,000 / Joint > \$342,000 to \$410,000	\$527.50
Individual > \$205,000 to < \$500,000 / Joint > \$410,000 to < \$750,000	\$649.20
Individual >= \$500,000 / Joint >= \$750,000	\$689.90

Roth conversion window

Physicians often have a conversion window after retirement but before RMDs and Social Security. During this period, partial Roth conversions may convert future taxable RMDs into Roth assets. The analysis should include current bracket, future RMDs, survivor tax filing status, IRMAA thresholds, charitable intent, and estate objectives.

Social Security planning

Social Security should be evaluated as part of a retirement-income plan, not in isolation. Physicians with long high-income careers often qualify for significant benefits, but timing decisions should reflect longevity, spouse benefits, survivor benefits, work plans, tax cost, investment withdrawals, and Medicare premiums.

Retirement-income planning sequence

- Project after-tax retirement spending need.
- Map tax-deferred, Roth, taxable, HSA, real estate, pension, and business-sale assets.
- Model Roth conversions before RMDs and Medicare IRMAA where appropriate.
- Coordinate charitable giving, QCDs, and estate goals.
- Review annually because income, brackets, premiums, and health needs change.

13. Estate, Gift, Asset Protection, and Legacy Planning

Estate planning for physicians should protect family decision-making, preserve assets, prepare for incapacity, coordinate beneficiary designations, and transfer wealth efficiently. The legal documents are necessary, but the ongoing coordination is where planning often breaks down.

2026 federal estate planning context

For estates of decedents dying in 2026, the federal basic exclusion amount is \$15,000,000. High-net-worth physicians, practice owners, and clients with concentrated real estate, business value, or large retirement accounts should review estate planning while the exemption is known. [S1]

Essential documents to review

- Will and/or revocable living trust.
- Durable financial power of attorney.
- Health care power of attorney and advance directive.
- HIPAA authorization.
- Guardian provisions for minor children.
- Beneficiary designations for retirement accounts, life insurance, annuities, and transfer-on-death accounts.
- Buy-sell agreement for practice owners.
- Entity documents, operating agreements, shareholder agreements, and employment agreements.

Asset protection

Physicians face professional risk, business risk, creditor exposure, cyber/privacy risk, and lifestyle-related liability. Asset protection planning should be done prospectively, while solvent, and with legal counsel. Core planning may include malpractice insurance, umbrella liability coverage, entity separation, retirement accounts, trust planning, titling review, and contractual risk management.

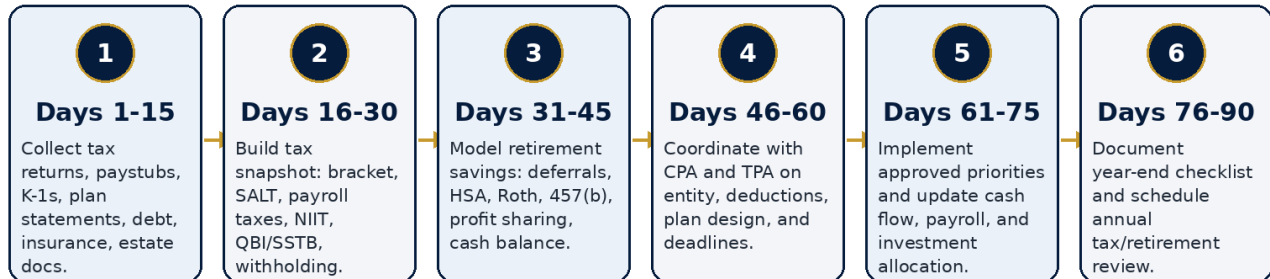
Practice transition and exit planning

A physician practice may represent a major component of net worth. Exit planning should address valuation, buyer readiness, associate agreements, succession, real estate, tax structure of sale, goodwill, restrictive covenants, retention incentives, and post-sale income planning.

14. The 90-Day Physician Planning Sprint

A guide is useful only if it produces action. The 90-day sprint below turns the planning concepts into a process that can be implemented with an advisor, CPA, TPA, attorney, and employer benefits team.

90-Day Physician Planning Sprint



90-day action plan

Period	Theme	Work to Complete
Days 1-15	Gather documents	Tax returns, W-2s, K-1s, 1099s, paystubs, plan statements, student loan data, insurance, estate documents.
Days 16-30	Tax snapshot	Bracket, SALT, AMT, payroll tax, NIIT, withholding, estimated taxes, QBI/SSTB, deductions.
Days 31-45	Retirement model	Max deferrals, HSA, IRA/backdoor Roth, 457(b), profit sharing, cash balance/defined benefit.
Days 46-60	Professional coordination	CPA, TPA, attorney, payroll provider, custodian, benefits department.
Days 61-75	Implementation	Adopt changes, update payroll, fund accounts, document strategy, adjust investments.
Days 76-90	Review and monitoring	Set tax calendar, review dates, annual wealth review, next-year planning items.

Client Call to Action

Schedule a Physician Tax & Retirement Strategy Call to review your current tax picture, retirement plan options, and priority next steps. Insert scheduling link here after compliance approval.

15. Checklists, Data Request, and Source Notes

Physician data request checklist

- Most recent two years of federal and state tax returns.
- Current-year paystubs for physician and spouse, including bonus and deferred compensation details.
- W-2, 1099, and K-1 documents.
- Retirement plan statements and plan summaries for 401(k), 403(b), 457(b), SEP, SIMPLE, pension, cash balance, or defined benefit plans.
- IRA, Roth IRA, HSA, brokerage, bank, and 529 statements.
- Student-loan data from StudentAid.gov and private lenders.
- Employer benefits guide, open-enrollment summary, and disability insurance details.
- Life, disability, long-term care, umbrella, malpractice, and business insurance policies.
- Estate documents, beneficiary designations, trust documents, and powers of attorney.
- Practice financial statements, entity documents, payroll reports, retirement plan documents, and buy-sell agreements for owners.

Annual planning calendar

Period	Planning Actions
January-February	Gather prior-year documents; verify contribution deadlines; review 1099s and K-1 expectations.
March-April	Tax filing review; identify tax return lessons; update estimated taxes.
May-June	Midyear tax projection; review retirement funding pace and charitable plan.
July-August	Practice owner plan design; cash balance or profit-sharing modeling; employee-benefit review.
September-October	Open enrollment; student-loan recertification review; Roth conversion modeling.
November-December	Finalize year-end deductions, contributions, charitable giving, withholding, and implementation tasks.

Source notes

Ref.	Source
S1	IRS, "IRS releases tax inflation adjustments for tax year 2026, including amendments from the One, Big, Beautiful Bill," IR-2025-103, Oct. 9, 2025. https://www.irs.gov/newsroom/irs-releases-tax-inflation-adjustments-for-tax-year-2026-including-amendments-from-the-one-big-beautiful-bill
S2	IRS, "401(k) limit increases to \$24,500 for 2026, IRA limit increases to \$7,500," IR-2025-111. https://www.irs.gov/newsroom/401k-limit-increases-to-24500-for-2026-ira-limit-increases-to-7500
S3	IRS, "COLA increases for dollar limitations on benefits and contributions," updated June 9, 2026. https://www.irs.gov/retirement-plans/cola-increases-for-dollar-limitations-on-benefits-and-contributions
S4	IRS, "Correction to state and local income tax deduction amount in the 2026 Form 1040-ES," updated March 16, 2026. https://www.irs.gov/forms-pubs/correction-to-state-and-local-income-tax-deduction-amount-in-the-2026-form-1040-es
S5	IRS, Revenue Procedure 2025-19, 2026 HSA inflation-adjusted amounts. https://www.irs.gov/pub/irs-drop/rp-25-19.pdf
S6	IRS Topic No. 506, Charitable Contributions, updated 2026. https://www.irs.gov/taxtopics/tc506
S7	IRS, Qualified business income deduction. https://www.irs.gov/newsroom/qualified-business-income-deduction
S8	IRS, About Form 8995-A and 2026 draft Form 8995-A materials. https://www.irs.gov/forms-pubs/about-form-8995-a
S9	IRS Topic No. 560, Additional Medicare Tax, updated June 1, 2026. https://www.irs.gov/taxtopics/tc560
S10	IRS, Net Investment Income Tax, updated July 1, 2026. https://www.irs.gov/individuals/net-investment-income-tax
S11	IRS, Retirement topics - Required minimum distributions (RMDs), updated 2026. https://www.irs.gov/retirement-plans/plan-participant-employee/retirement-topics-required-minimum-distributions-rmds
S12	CMS, "2026 Medicare Parts A & B Premiums and Deductibles," Nov. 14, 2025. https://www.cms.gov/newsroom/fact-sheets/2026-medicare-parts-b-premiums-deductibles
S13	SSA, "What is the current maximum amount of taxable earnings for Social Security?" Jan. 2, 2026. https://www.ssa.gov/faqs/en/questions/KA-02387.html
S14	Federal Student Aid, Public Service Loan Forgiveness Form and Help Tool. https://studentaid.gov/pslf/
S15	IRS Notice 2025-67, 2026 Amounts Relating to Retirement Plans and IRAs, including QCD indexed limit. https://www.irs.gov/pub/irs-drop/n-25-67.pdf
S16	IRS, "Updates to frequently asked questions about educational assistance programs," FS-2026-10.

	https://www.irs.gov/newsroom/updates-to-frequently-asked-questions-about-educational-assistance-programs
S17	Federal Student Aid, IDR Plan Court Actions: Impact on Borrowers, updated July 1, 2026. https://studentaid.gov/announcements-events/idr-court-actions
S18	IRS, "Treasury, IRS issue guidance on the additional first year depreciation deduction amended as part of the One Big Beautiful Bill," IR-2026-06. https://www.irs.gov/newsroom/treasury-irs-issue-guidance-on-the-additional-first-year-depreciation-deduction-amended-as-part-of-the-one-big-beautiful-bill
S19	IRS Publication 946, "How To Depreciate Property," 2025 edition with 2026 Section 179 updates. https://www.irs.gov/publications/p946
S20	IRS, Working Families Tax Cuts, updated July 6, 2026. https://www.irs.gov/newsroom/working-families-tax-cuts

Closing Page

Next Step

Use this guide to begin a structured Physician Tax & Retirement Strategy Review. The next step is to gather the key documents, complete the planning checklist, and identify the highest-priority opportunities before the next tax deadline.

Elite Physician Wealth Planning™

Powered by M A Epps & Associates

Michael A. Epps, ChFC®, RICP® | Founder & Chief Wealth Strategist

[Insert firm phone] | [Insert firm email] | [Insert scheduling link]

Educational guide. Not tax, legal, investment, or accounting advice. Review with qualified professionals before implementation.